



## POLICY

### Communication with General Practitioners (GP) and Shared Care

|                       |                                                   |
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| <b>Scope (Staff):</b> | All NMHS Mental Health (MH) Adult Community Staff |
| <b>Scope (Area):</b>  | All NMHS MH Adult Community Services              |

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## 1. Aim

Referring and treating GPs will be provided with relevant and timely information regarding consumer mental health assessment, treatment and management in order to promote effective, efficient and continuing shared care with NMHS MH Adult Community Mental Health Service clinicians. GPs will be collaborated with to ensure that physical health needs are addressed and managed in line with consumer centred care.

## 2. Background

The [MHPHDS Mental Health Care Coordination Framework \(2011\)](#) outlines the following Process Elements to Care Coordination:

- When the services provided by one agency are significantly inter-dependent on the services provided by another agency, a more involved level of coordinated care is required.
- Clear inter-agency communication is required to coordinate the consumers care. Agencies rely on exchange of information about the interventions that are aimed at

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supporting each consumer. It also supports service delivery programs provided by other agencies.

- Care coordination requires agencies to work collaboratively to help meet the needs of people with long term mental health disorders that one agency cannot address alone.

Effective communication and good clinical handover of consumer information is critical to safe, well-integrated care. In a community setting, GPs provide coordination of all aspects of a consumers' health care and are a key member of a consumers' healthcare team.

The [National Safety and Quality Health Service Standards \(Version 2\)](#) require all health care services to communicate for safety and provide comprehensive care. The [Chief Psychiatrist's Standards for Clinical Care](#) also outline the responsibility of mental health services to ensure that the physical health needs of those they care for are well managed.

The delivery of comprehensive care is based on partnering with consumers, carers and families to identify, assess and manage consumers clinical needs.

### 3. Risk

If the policy is not followed, important health information may not be shared between community mental health services and the consumer's GP

### 4. Definitions

**Private Mental Health Clinician:** Any clinician (psychiatrist, psychologist, GP etc) involved in the consumers care and working in the private sector and is engaged in a shared care arrangement with the public mental health service.

**Shared care arrangement:** When a public mental health team (one or more clinicians) collaboratively manages a consumer in partnership with another stakeholder.

**Consumer:** An Individual living with a mental health concern and engaged with MHPHDS Community Mental Health service.

**Carer/family member:** A person who supports a consumer.

**Nominated person:** Assists the consumer to exercise their rights and can help represent the consumers views and preferences.

**Guardian:** Guardians make personal, lifestyle and treatment-related decisions in the best interests of a person with a decision-making disability. Guardianship orders specify the areas in which the guardian can make decisions.

### 5. Principles

1. MHPHDS Adult Community Mental Health clinicians recognise the value of effective communication with other health professionals, including primary healthcare providers, as best practice service delivery.

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2. MHPHDS Adult Community Mental Health clinicians seek consumer consent to communicate information to other health professionals and respect the consumers wishes should they refuse consent unless risk issues require communication to take place.
  3. MHPHDS Adult Community Mental Health clinicians inform consumers of the potential implications of not providing consent for communication between health professionals.
  4. MHPHDS Adult Community Mental Health clinicians communicate with other health professionals in a timely manner, which is essential for consumer care. The degree of urgency will determine the communication medium and timeframe that is most appropriate.
  5. MHPHDS Adult Community Mental Health clinicians are aware of the [MH Physical Healthcare Management Policy](#) and their responsibility in regard to communicating with GPs relating to this.

## 6. Procedure

### 6.1 Maintaining GP Details

On initial assessment or contact, the assessing clinicians should check the GP contact details and obtain consent from the consumer to communication with their GP. Consent should be documented on the Consent to Release/Receive Information form (PMR 3) kept in the clinical file. The GP details should be updated onto PSOLIS and onto the Personal Support Person form in the clinical file (PMR 2).

GP details should be checked three monthly (in line with clinical reviews) with the consumer. If the consumer notifies the case manager or another clinical team member of a change to their GP details these should be updated onto PSOLIS and on the PMR 2, and the old GP details expired. Consent will be discussed at each change of GP and updated information recorded on the PMR 3.

### 6.2 Communication with the GP

The consumer can be given a copy of all letters sent to the GP at the discretion of the treating team. The consumer will be provided with an updated Treatment Support and Discharge plan. Where a response is required from the GP by the CMH -following communication, the treating team should liaise with the GP to ensure the information required is received.

A [Referral Acknowledgement Letter](#) is sent to the referring or treating GP as soon as possible after the relevant intake meeting, acknowledging any referrals received and indicating the appropriate team member and associated appointment date or anticipated waiting period (a similar letter is also sent to the consumer). If a referral is declined, a [Referral Declined Letter](#) should provide alternative management options to the GP.

A [Physical Health Letter](#) should be sent in line with the [MHPHDS Mental Health Physical Healthcare Management Policy](#). All MHPHDS Mental Health Adult Community clinicians should refer to the [Physical Healthcare Management Policy](#) for further detail and to access physical healthcare letter templates.



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An [Assessment and Treatment Plan Letter](#) must be sent to the referring or treating GP after the first contact with the consumer. This letter will briefly outline assessment/diagnosis, treatment plan, probable length of treatment, medications prescribed, identified risks and other relevant information. Particular emphasis should be placed on providing information that will enable the GP to participate in the management or shared care of the consumer. Where there is a treating psychiatrist, this letter will come from them with input from the case manager. In the absence of a doctor's assessment, the case manager is responsible for the assessment and treatment plan letter.

An [Outcome of Consumer Review and Management Plan Letter](#) outlining the consumers progress and continuing care needs must be sent to the GP at regular intervals and at a minimum every six months. The letter should be sent within one week of the clinical review and provide the GP with an update on the review and any changes to the management plan. Where the GP is part of the management plan, a copy of the management plan should be sent with the letter. These letters are the responsibility of the treating doctor but can be delegated to a clinician in the treating team with mutual consent. Discussion between the treating Doctor and a clinician in the treating team is to occur regarding any changes to medication, diagnosis and goals and development of co-signed letter, see outcome of consumer review letter template.

Outside of the three monthly review letter A [Letter](#) will be sent, or a [Telephone Call](#) will be made, by the treating doctor but can be delegated to a clinician in the treating team with mutual consent notifying the treating GP of any significant changes to the treatment plan, medication, etc., that the GP needs to be aware of in order to make an effective contribution to the management or shared care of the consumer.

A [Final \(Discharge\) Letter](#) must be sent at the conclusion of treatment outlining the continuing care needs of the consumer and the mechanism for re-referral to the mental health service should this be necessary. If attempts to follow up a referral have not been successful in engaging the consumer a [Discharge \(Patient did not engage\) Letter](#) should be sent to the GP providing a brief summary of the attempts made to assess/engage the consumer. These letters are the responsibility of the treating doctor but can be delegated to a clinician in the treating team with mutual consent.

### 6.3 Shared Care Arrangements for Public and Private Mental Health Clinicians

The following are guiding principles to assist in shared care arrangements between public and private mental health clinicians:-

- A formal agreement or arrangement can assist to delineate the roles, functions and areas of clinical focus for each health care professional, increase communication and assist them to work collaboratively to optimize treatment in the best interest of the consumer.
- All parties to shared care agreement should commit to communicate regularly, verbally or in writing, at a frequency that matches the clinical need.
- Significant changes in the consumers presentation as outlined below will be communicated in a timely manner,
  - Management (e.g. medication, modality of psychological intervention)



- Mental status assessment
- Risk of self-harm or harm to others
- Level of cooperation with the treatment plan or if the consumer has ceased to attend treatment.
- There must be open communication and engagement with the consumer/ carer/ nominated support person. There must be evidence of consumer and carer engagement.

## 7. Compliance and Evaluation

This policy will be monitored through the annual GP Communication Audit. The GP Liaison Officer in each community catchment may conduct local audits to review compliance with the policy. The annual Documentation Audit also audits some communication with GPs.

### Related internal policies, procedures and guidelines

[MHPHDS Mental Health Care Coordination Framework \(2011\)](#)  
[MHPHDS Mental Health Case Management Policy](#)  
[MHPHDS Mental Health Clinical Record Documentation Policy](#)  
[MHPHDS Mental Health Discharge Planning For Community Patients Policy \(Currently Under Review\)](#)  
[MHPHDS Mental Health Electronic Transmission of Patient Information Guideline](#)  
[MHPHDS Mental Health Physical Healthcare Management Policy](#)  
[NMHS Healthcare Record – Access and Document Release Policy](#)

### References

[Chief Psychiatrist's Standards for Clinical Care](#)  
['Triage to Discharge' – Mental Health Framework for State-wide Standardised Clinical Documentation](#)  
[National Safety and Quality Health Service Standards \(Version 2\)](#)  
[WA Department of Health Clinical Handover Policy](#)

### Useful resources

PMR 2 Personal Support Person (Including Substitute Decision Makers) Form  
 PMR 3 Consent to Release/Receive Information Form  
[NMHS MHPHDS Mental Health Referral Acknowledgement Letter to General Practitioners](#)  
[NMHS MHPHDS Mental Health Referral Declined Letter to General Practitioners](#)



[NMHS MHPHDS Mental Health Assessment and Treatment Plan Letter to General Practitioners](#)  
[NMHS MHPHDS Outcome of Patient Review and Management Plan Letter to General Practitioners](#)  
[NMHS MHPHDS Discharge \(Treatment Complete\) Letter to General Practitioner](#)  
[NMHS MHPHDS Discharge \(Patient Did Not Engage\) Letter to General Practitioners](#)

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| Policy Sponsor                                                                                    | Community Co Director of Mental Health MHPHDS                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |                |            |              |            |
| Policy Contact                                                                                    | GP Liaison Officers MHS                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                |            |              |            |
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| Approved by:                                                                                      | Community Adult Mental Health Leadership Committee                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |                |            | Date:        | 24/07/2024 |
| Endorsed by:                                                                                      | Community Co Director of Mental Health                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |                |            | Date:        | 22/08/2024 |
| NSQHS Standards Applicable:                                                                       | <div style="display: flex; justify-content: space-between;"> <div style="width: 48%;"> <input type="checkbox"/>  Std 1: Clinical Governance<br/> <input checked="" type="checkbox"/>  Std 2: Partnering with Consumers<br/> <input type="checkbox"/>  Std 3: Preventing and Controlling Healthcare Associated Infection<br/> <input type="checkbox"/>  Std 4: Medication Safety </div> <div style="width: 48%;"> <input checked="" type="checkbox"/>  Std 5: Comprehensive Care<br/> <input checked="" type="checkbox"/>  Std 6: Communicating for Safety<br/> <input type="checkbox"/>  Std 7: Blood Management<br/> <input type="checkbox"/>  Std 8: Recognising and Responding to Acute Deterioration </div> </div> |                |            |              |            |
| Chief Psychiatrist's Standards for Clinical Care:                                                 | Aboriginal Practice, Assessment,<br>Care Planning,<br>Consumer and Carer Involvement in Individual Care,<br>Physical Health Care of Mental Health Consumers,<br>Risk Assessment and Management,<br>Transfer of Care                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |                |            |              |            |
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| Document Version Control |         |                                |                                                           |
|--------------------------|---------|--------------------------------|-----------------------------------------------------------|
| Date                     | Version | Author                         | Notes                                                     |
| 19/10/2022               | V1.0    | Co-Director Adult Community MH | Initial endorsement.                                      |
| 22/08/2024               | V1.1    | Co-Director Adult Community MH | Reviewed and updated in accordance with current practice. |

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative. (ISD number 1760)

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